COMPOUNDING CLAIMS · RED FLAG REPORT

# $1,200 Cream. $47 Alternative.

*A six-flag walkthrough for spotting compounding claims that warrant a closer look. The pattern is consistent: high-cost multi-ingredient compounds with commercial alternatives available the entire time. The system processed it. Nobody flagged it.*

## 01 Six Red Flags to Apply

**Data pull first:** request a compounding-specific claims extract from your PBM covering the past 12 months. Need all claims with a compounding indicator, dispensing pharmacy name and NPI, prescriber name and NPI, ingredient lists for multi-ingredient compounds, total claim cost with ingredient cost breakdown, and therapeutic category. Sort by total cost, highest first; focus on the top 20% by dollar amount. Apply each flag below to those high-cost claims.

### Flag 1 · High cost per claim

Threshold: > $500 / claim

Individual compounding claims exceeding $500 warrant review. Most legitimate compounds cost substantially less than that. The high-cost outliers are where the problem patterns live.

- Filter claims to compounds with total cost above $500
- Rank flagged claims by total dollars to prioritize review

### Flag 2 · Multi-ingredient topicals

Threshold: 4+ active ingredients

Creams or ointments with four or more active ingredients. The most common vehicle for inflated pricing. Clinical evidence for multi-ingredient topical combinations is limited, and that is a pharmacist's read, not a marketing one.

- Identify topical compounds with 4+ active ingredients
- Confirm clinical rationale for each combination is documented

### Flag 3 · Pharmacy concentration

Single pharmacy disproportionate share

If a single pharmacy generates a disproportionate share of your compounding claims, that concentration suggests marketing or incentive relationships rather than organic patient need. We see this constantly in our reviews.

- Group compounding claims by dispensing pharmacy NPI
- Flag any single pharmacy with disproportionate compound volume

### Flag 4 · Prescriber concentration

Single prescriber disproportionate share

A single prescriber generating a disproportionate share of compounding referrals may indicate a prescriber-pharmacy financial relationship. Worth investigating before paying the next claim.

- Group compounding claims by prescriber NPI
- Flag prescribers driving unusual compounding referral volume

### Flag 5 · Commercial alternative available

FDA-approved product exists at fraction of cost

The most straightforward flag. If an FDA-approved product exists for the same indication at a fraction of the cost, the clinical justification for the compound should be documented and defensible. In our experience, usually it is not.

- Identify primary active ingredient in each flagged compound
- Search FDA-approved commercial products with same ingredient
- Document the cost differential per claim
- Confirm clinical rationale for compound vs. commercial

### Flag 6 · Rapid spend growth

`Quarter-over-quarter without population change`

Compounding costs increasing significantly quarter over quarter without corresponding population changes. Often signals a new pharmacy or prescriber entering your network with an aggressive utilization pattern.

- Plot quarterly compounding spend over the past 8 quarters
- Cross-reference growth with new pharmacies or prescribers
- Identify quarters with material growth

*Illustrative red flag thresholds for educational purposes. Specific cost thresholds and concentration ratios should be calibrated to your plan size and population. Compounding can serve legitimate clinical needs; the flags identify claims worth a closer look, not automatic denials.*

## 03 What "Normal" Looks Like

The flags identify outliers; you also need a baseline that says what the data should look like in a healthy plan. For most self-funded plans, compounding sits within these parameters. Material deviation from any of them is a problem worth investigating.

VOLUME

**Compounding is a small percentage of total pharmacy claims.**

By claim count, legitimate compounding sits in single-digit-percentage territory of total pharmacy volume. Plans where compounding has grown into a meaningful share of total claims usually have an upstream pharmacy or prescriber concentration driving it.

AVERAGE COMPOUND COST

**Typically under $200 per claim for legitimate compounds.**

Average compound cost above $200-$300 suggests the plan's compounding mix is weighted toward high-cost multi-ingredient claims. The Flag 1 threshold ($500 per claim) catches the worst outliers; the average is the population-level signal.

PHARMACY AND PRESCRIBER DISTRIBUTION

**No single pharmacy or prescriber dominates compounding volume.**

Healthy distributions show compounding spread across multiple pharmacies and prescribers in proportion to overall network share. Single-source concentration is the strongest pattern signal in the entire flag set.

COMMERCIAL ALTERNATIVE USE

**Commercial alternatives are used when available.**

When an FDA-approved product exists at a fraction of the compound cost, healthy plans show meaningful uptake of the commercial alternative. Plans where the compound dominates despite an alternative being available are missing a clinical-review step somewhere upstream of dispensing.

## 04 Action Plan and Contract Asks

Findings without action are observations. Build the plan from your flagged claims, then close the contract gaps that let the pattern develop in the first place. Many PBM contracts treat compounding as an afterthought; that gap is fixable at the next renewal redline.

§ **Flag high-cost compounds with available alternatives for clinical review**
Submit each flagged claim that meets Flag 5 to your clinical advisory team for documented review. Decisions on go-forward coverage should be on file with rationale.

§ **Implement quantity limits or PA above a cost threshold**
Set a dollar threshold for compounding that requires prior authorization. The threshold itself is a meaningful filter; pharmacies that cannot document clinical rationale will route patients to commercial alternatives.

§ **Push your PBM to implement compounding edits**
Ingredient-level cost caps, automatic commercial-alternative checks at adjudication, and quarterly compounding utilization reports as a standing deliverable. Most PBMs have these capabilities; few enable them by default.

§ **Add compounding-specific contract language at next renewal**

Define utilization controls, pricing methodology, credentialed-pharmacy criteria, separate compounding reporting, and audit rights with full access to verify appropriateness. Contract gaps are how compounding becomes a problem in the first place.

§ **Schedule quarterly compounding claims review**

Put it on the benefits committee calendar as a standing quarterly item, not an ad hoc audit. Track flag findings over time and close the loop on prior-quarter actions.